WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 10

Psychotherapy for Special Populations

Children, Adolescents, Elderly, Psychosis, and Cultural Considerations

Child Therapy · Adolescent DBT · CBTp · Cultural Formulation · Perinatal · Trauma

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Deep Study

Psychotherapy for Special Populations — Weave Psychotherapy Vol. 10

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1. CHILD AND ADOLESCENT THERAPY

1.1 Developmental Considerations

Children and adolescents are not miniature adults. Every therapeutic intervention must be filtered through the lens of developmental stage — cognitive capacity, emotional vocabulary, abstract reasoning ability, attachment organisation, and social context all shape what is possible in the therapy room.

Piaget's stages provide the scaffolding for therapy adaptation:

Stage	Age	Therapeutic Implication
Sensorimotor	0–2	Therapy is parent-directed; focus on attachment, parent-infant interaction
Preoperational	2–7	Concrete, egocentric thinking; use play, art, puppets; cannot reflect on own cognition
Concrete operational	7–11	Can follow rules, classify, reason about concrete events; CBT possible with simplification
Formal operational	12+	Abstract reasoning emerges; standard CBT adaptable; can engage in metacognition

EXAM PEARL

The capacity for metacognition (thinking about one's own thinking) does not reliably emerge until age 11–12. This is why standard CBT — which requires identifying and evaluating automatic thoughts — is difficult to deliver before this age. Younger children benefit more from behavioural strategies, play-based approaches, and parent-mediated interventions.

Key developmental principles for child therapy:

- **Therapeutic alliance looks different** — rapport with the child AND the parent system is essential. A child cannot self-refer, pay for sessions, or control their environment.
- **Informed consent** requires age-appropriate explanation and assent from the child plus legal consent from parents/guardians.
- **Confidentiality** must be negotiated carefully — children deserve privacy, but parents have a right to know about safety concerns. Establish clear rules at the outset.

- **Attention span** is shorter. Sessions are typically 30–45 minutes for younger children, 50 minutes for adolescents.
- **Externalising vs. internalising** presentations differ in engagement — externalising children may resist therapy (brought by frustrated parents), while internalising children may be compliant but emotionally avoidant.

1.2 Play Therapy

Play is the natural language of children. Where adults use words to communicate their inner world, children use symbolic play, art, and action. Therapeutic play allows the child to express conflicts, process trauma, and rehearse new relational patterns in a safe, contained environment.

Two major traditions:

Non-directive (Child-Centred) Play Therapy — Virginia Axline (1947)

- Based on Rogers' client-centred principles adapted for children
- Eight principles: establish warm rapport; accept the child completely; create permissiveness; recognise and reflect feelings; respect the child's ability to solve problems; let the child lead; therapy is gradual; only set limits for safety, reality, and responsibility
- Therapist follows the child's lead without interpretation
- Evidence base: moderate for internalising problems, anxiety, trauma in under-10s

Directive Play Therapy

- Therapist introduces structured activities with therapeutic goals
- Includes cognitive-behavioural play therapy (CBPT), theraplay, sandtray therapy
- CBPT (Knell, 1993): uses puppets, dolls, stories to teach cognitive restructuring, problem-solving, and coping to children aged 3–8

CLINICAL ANCHOR

Play therapy is not "just playing." A trained play therapist observes themes (control, aggression, nurturance, chaos), tracks metaphorical content, and uses the therapeutic relationship within play as the vehicle for change. When a child repeatedly crashes a toy ambulance, that is communication — not recreation.

1.3 CBT Adaptations for Children and Adolescents

Kendall's Coping Cat programme (Kendall et al., 1997) is the gold standard CBT protocol for childhood anxiety disorders (ages 7–13). It uses the FEAR acronym:

Step	Meaning
F	Feeling frightened? (recognise somatic and emotional cues)
E	Expecting bad things to happen? (identify anxious self-talk)
A	Actions and attitudes that can help (develop coping plan)
R	Results and rewards (self-evaluate and self-reinforce)

The programme proceeds in two phases: (1) skill-building (8 sessions of psychoeducation, relaxation, cognitive restructuring, problem-solving) and (2) practice/exposure (8 sessions of graded in-vivo exposure using a fear hierarchy). RCTs show 60–80% of children no longer meet diagnostic criteria at post-treatment, with gains maintained at 7-year follow-up (Kendall et al., 2004).

EXAM PEARL

Kendall's Coping Cat is the most studied CBT programme for child anxiety. The FEAR acronym and the two-phase structure (skill-building then exposure) are frequently examined. It has been adapted across cultures (e.g., Cool Kids in Australia by Rapee et al.).

CBT for adolescent depression — the Treatment for Adolescents with Depression Study (TADS, 2004) compared fluoxetine, CBT, combination, and placebo for adolescent major depression. Key finding: combination treatment (fluoxetine + CBT) was superior to either alone. CBT alone was not significantly better than placebo at 12 weeks but caught up by 36 weeks.

> EXAM STRATEGY
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> TADS is a landmark trial. Remember: at 12 weeks, combination > fluoxetine > CBT = placebo. At 36 weeks, all active treatments converge. CBT alone had the slowest onset but added protection against suicidality when combined with fluoxetine.

1.4 Parent Training Programmes

Parent training is the first-line psychosocial intervention for conduct problems in children aged 3–11 (NICE CG158). It works by modifying the coercive parent-child interaction cycles described by Patterson's social learning theory.

Programme	Developer	Age Range	Format	Key Features
Webster-Stratton Incredible Years	Carolyn Webster-Stratton	3–8	Group (12–20 sessions)	Video modelling, collaborative approach, pyramid (child, parent, teacher components), strongest evidence base for conduct disorder
Triple P (Positive Parenting Program)	Matt Sanders (Queensland)	0–16	5 levels of escalating intensity	Universal to targeted; population-level public health model; Level 4 (10 sessions) for clinical problems
Parent Management Training (PMT)	Alan Kazdin	3–13	Individual (12–20 sessions)	Based on operant conditioning; token economies; therapist works primarily with parents

Programme	Developer	Age Range	Format	Key Features
Parent-Child Interaction Therapy (PCIT)	Sheila Eyberg	2–7	Dyadic, live coaching via bug-in-ear	Two phases: CDI (child-directed interaction — like play therapy) then PDI (parent-directed interaction — commands and compliance); real-time coaching

KEY INSIGHT

MNEMONIC — WITP (parent training programmes): **W**ebster-Stratton (video + group), **I**ncredible Years = same, **T**riple P (5 levels, population model), **P**MIT/PCIT (operant/live coaching).

EXAM PEARL

PCIT is unique because the therapist coaches the parent in real-time through an earpiece while the parent interacts with the child. It has strong evidence for conduct problems AND child physical abuse prevention. The CDI phase maps onto attachment-enhancing interaction; the PDI phase maps onto behavioural management.

1.5 Adolescent DBT: The Rathus and Miller Model

Rathus and Miller adapted Linehan's standard DBT for multiproblem, suicidal adolescents at Montefiore Medical Center, producing the most comprehensive adolescent DBT model available.

The Biosocial Model in Adolescent Context

The transaction between biological vulnerability (high emotional sensitivity, high reactivity, slow return to baseline) and an invalidating environment produces chronic emotional dysregulation. In adolescents, this manifests as identity confusion, impulsivity, self-harm, interpersonal chaos, and family conflict. Critically, the model is **transactional** — teens can invalidate their parents, and parents may have their own biological vulnerabilities.

> CLINICAL ANCHOR
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> The biosocial model is not a blame model. Teaching it to families requires emphasising that neither party caused the problem alone — it is the fit between the adolescent's temperament and the environment's response that creates dysregulation. The "fire in a fireplace" metaphor works well: once the fire is burning (biological sensitivity), adding small logs (daily stressors) reignites it quickly.

Five Modules (vs. Four in Standard Adult DBT)

Module	Problem Area Addressed	Duration
Core Mindfulness	Confusion about self, reduced awareness	2 weeks (repeated before each module)
Distress Tolerance	Impulsivity, crisis behaviours	4 weeks
Walking the Middle Path	Adolescent-family dialectical conflicts	4 weeks
Emotion Regulation	Emotional dysregulation	4 weeks
Interpersonal Effectiveness	Interpersonal problems	4 weeks

Walking the Middle Path is the fifth module unique to adolescent DBT, addressing:

1. **Dialectics** — moving from "either/or" to "both/and" thinking
2. **Validation** — six levels (Linehan's hierarchy: listening, reflection, mind-reading, validation by past history, validation by current context, radical

genuineness)

3. **Behaviour change** — positive/negative reinforcement, shaping, extinction, punishment

Three Adolescent-Specific Dialectical Dilemmas

Dilemma	Loose Pole	Strict Pole	Middle Path
Leniency vs. Authoritarian Control	No rules, no monitoring	Rigid rules, extreme consequences	Authoritative parenting — firm limits with warmth
Normalising Pathological vs. Pathologising Normative	Minimising self-harm ("everyone does it")	Treating normal teen behaviour as dangerous	Use developmental norms as guide
Forcing Autonomy vs. Fostering Dependence	Cutting support prematurely	Helicopter parenting	"Holding on while letting go"

Key Structural Adaptations

- **Multifamily skills group** — parents attend alongside adolescents (3–5 families, 2–hour sessions)
- **Phone coaching extended to parents**
- **24–week cycle** (vs. one year in adult DBT)
- **Simplified language**, developmentally appropriate examples
- **TIPP skills** for crisis management (Temperature, Intense exercise, Paced breathing, Progressive relaxation)

> EXAM PEARL
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> The TIPP skills are the "emergency kit" of adolescent DBT. Temperature (cold water on face) activates the mammalian dive reflex, rapidly reducing heart rate via parasympathetic activation. This is the fastest skill in the DBT toolkit — effective within 30 seconds.

Evidence Base

The Mehlum et al. (2014) Norwegian RCT is the strongest evidence: 16–week DBT with multifamily format vs. enhanced usual care produced significant reductions in self-harm and suicidal ideation in adolescents. Additional quasi-experimental studies support DBT for adolescents with BPD features, bipolar disorder, eating disorders, and externalising disorders.

> EXAM STRATEGY
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> For adolescent self-harm, DBT (Rathus-Miller model) has the strongest evidence. For adolescent depression, CBT + SSRI (TADS model). For adolescent anxiety, CBT (Coping Cat/Cool Kids). For conduct disorder in younger children, parent training (Webster-Stratton, Triple P, PCIT).

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2. THERAPY FOR OLDER ADULTS

2.1 Age-Related Considerations

Therapy with older adults (typically defined as 65+) requires adaptation across multiple domains: cognitive changes, sensory decline, cohort effects, medical comorbidity, loss and bereavement, and existential concerns about mortality and legacy.

Cognitive changes with ageing:

- Processing speed declines
- Working memory capacity reduces
- Episodic memory retrieval slows
- Crystallised intelligence (vocabulary, general knowledge) is preserved or improves
- Executive function may decline, particularly set-shifting and inhibition

Practical adaptations:

- Slower pacing, shorter sessions if needed

- Written summaries and handouts (larger font)
- Repetition and rehearsal of key points
- Involve carers/family where appropriate
- Check hearing and vision; adjust seating, lighting, volume
- Address transportation, mobility, and scheduling barriers

> EXAM PEARL
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> Older adults are consistently underrepresented in psychotherapy research and underreferred for psychological treatment. The assumption that "you can't teach an old dog new tricks" is empirically wrong — CBT, IPT, and problem-solving therapy all show efficacy in late-life depression comparable to outcomes in younger adults.

2.2 Laidlaw's Comprehensive Conceptualisation (CCCF) Model

Ken Laidlaw developed the **Comprehensive Conceptualisation Framework for CBT with Older Adults** (Laidlaw, 2015), which augments the standard Beckian case formulation with four cohort-specific factors:

Factor	Description	Example
Cohort beliefs	Generational attitudes shaped by historical context	"You don't talk about feelings"; "Mental illness is shameful"; wartime stoicism
Role transitions and investments	Losses of occupational, social, and family roles	Retirement, empty nest, widowhood, becoming a carer, loss of driving licence
Intergenerational linkages	Relationships with younger generations; legacy concerns	Grandparenting role, family conflict, dependency anxieties
Socio-cultural context	Ageism, social isolation, poverty, institutional living	"I'm just a burden"; internalised ageism; reduced social network

KEY INSIGHT

MNEMONIC — CRIS (Laidlaw's four factors): **C**ohort beliefs, **R**ole transitions, **I**ntergenerational linkages, **S**ocio-cultural context.

CLINICAL ANCHOR

Laidlaw's CCCF model does not replace the standard CBT formulation — it wraps around it. The hot cross bun (thoughts-feelings-behaviours-physiology) sits at the centre, but the four additional factors provide the context that makes the formulation meaningful for an older person. Without them, the therapist risks pathologising normal ageing or missing the social determinants of distress.

2.3 Life Review Therapy and Reminiscence

Life review therapy (Butler, 1963) is a structured therapeutic intervention based on Erikson's eighth stage of psychosocial development: **ego integrity vs. despair**. The goal is to help older adults integrate their life experiences into a coherent, meaningful narrative, resolving regrets and affirming identity.

Types:

- **Simple reminiscence** — unstructured sharing of memories; improves well-being and socialisation
- **Life review** — structured, therapist-guided evaluation of the whole life span; targets unresolved conflicts, guilt, and incomplete mourning
- **Life review therapy** — formally structured therapeutic protocol; may use timelines, photographs, writing, or audio recording

Evidence: Cochrane review (Huang et al., 2015) found life review/reminiscence significantly reduced depressive symptoms in older adults, with moderate effect sizes. It is particularly suitable for care home residents and those with mild cognitive impairment.

> EXAM PEARL
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> Life review therapy is not "just chatting about the past." It is based on Erikson's ego integrity vs. despair stage and has Cochrane-level evidence for reducing depression in older adults. It is the one therapy specifically designed for the existential tasks of later life.

2.4 Bereavement and Loss

Older adults face cumulative losses: spouse, friends, siblings, health, independence, cognitive capacity, home, and social role. Most bereavement follows a natural course of adjustment. Complicated grief (prolonged grief disorder — now in ICD-11 and DSM-5–TR) requires intervention when grief remains intense, pervasive, and functionally impairing beyond 6–12 months.

Complicated Grief Treatment (CGT — Shear et al., 2005):

- Integrates CBT, IPT, and motivational interviewing techniques
- Dual process model (Stroebe & Schut, 1999): oscillation between loss-oriented and restoration-oriented coping
- Imaginal revisiting of the death narrative (exposure component)
- Situational revisiting of avoided activities/places
- Evidence: RCT showed CGT superior to IPT for complicated grief

2.5 Capacity Issues

When working with older adults, therapists must assess capacity to consent to treatment. In England and Wales, the Mental Capacity Act (2005) presumes capacity unless demonstrated otherwise. Capacity is decision-specific and time-specific.

> EXAM STRATEGY
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> In viva questions on older adult therapy, always mention: (1) Laidlaw's CCCF model for adapted CBT, (2) life review therapy based on Erikson, (3) the importance of screening for cognitive impairment (using MoCA/MMSE) before starting cognitively demanding therapy, and (4) the need to address practical barriers (transport, hearing, vision, carer involvement).

3. THERAPY FOR PSYCHOSIS

3.1 CBT for Psychosis (CBTp)

CBT for psychosis is the most evidence-based psychological intervention for schizophrenia-spectrum disorders and is recommended by NICE (CG178, 2014) for all people with psychosis or schizophrenia. The approach was developed primarily by Kingdon and Turkington (1994) and Morrison (2001).

The Normalising Approach

The foundation of CBTp is the **normalising rationale**: psychotic experiences exist on a continuum with normal experience. Hearing voices, paranoid thinking, and unusual beliefs are not categorically different from common experiences like hearing your name called when alone, suspecting others are talking about you, or holding superstitious beliefs.

> EXAM PEARL
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> The normalising rationale is central to CBTp. It reduces stigma, shame, and hopelessness. It does NOT mean telling the patient their experiences are "normal" — it means demonstrating that these experiences lie on a continuum and that stress, sleep deprivation, isolation, and trauma can push anyone along that continuum.

Working with Delusions

CBTp does not aim to eliminate delusions by frontal assault. Instead, the approach is collaborative and Socratic:

1. **Build a shared formulation** — understand the delusion's development in context (trauma, isolation, life events)
2. **Identify maintaining factors** — safety behaviours, confirmation bias, social isolation
3. **Peripheral questioning** — start with less emotionally charged beliefs before approaching core delusions
4. **Examine evidence** — "What makes you believe this? Is there any other explanation?"
5. **Behavioural experiments** — test predictions arising from delusional beliefs
6. **Reality testing** — gentle, collaborative, never confrontational

> CLINICAL ANCHOR
>
> Never directly challenge a delusion. The therapeutic stance in CBTp is one of "curious uncertainty" — neither agreeing with the delusion nor arguing against it. The therapist holds the position: "I can see this feels very real to you. I'm interested in understanding it better. Could we look at it together?"

Working with Hallucinations

Technique	Description
Normalising	Explain prevalence of voice-hearing in general population (5–15%)
Formulation	Understand the voice in context — when did it start? Whose voice? What triggers it?
Beliefs about voices	Address omnipotence ("the voice is all-powerful"), omniscience ("the voice knows everything"), compliance beliefs ("I must obey") — Chadwick & Birchwood (1994)
Coping strategy enhancement	Build on existing coping; add new strategies (distraction, focusing, subvocalisation)
Reattribution	Help the patient consider that the voice may be internally generated
Behavioural experiments	Test voice predictions (e.g., "the voice says if I go out, something bad will happen" — test this)

Behavioural Experiments for Paranoia

Freeman et al. (2015) developed a cognitive model of persecutory delusions emphasising the role of safety behaviours. Dropping safety behaviours (avoidance, hypervigilance, checking) through graded behavioural experiments is the primary intervention.

Steps:

1. Identify the threat belief and safety behaviour
2. Predict what will happen without the safety behaviour
3. Drop the safety behaviour (with therapist support)
4. Evaluate the outcome against the prediction
5. Update the belief

EXAM PEARL

Freeman's cognitive model of paranoia identifies six maintenance factors: anxiety processes, worry, negative self-beliefs, sleep disturbance, reasoning biases (jumping to conclusions), and safety behaviours. The jumping-to-conclusions bias (JTC) is measured by the beads task — individuals with paranoia make decisions based on less evidence than controls.

3.2 Family Intervention for Schizophrenia

Family intervention is recommended by NICE for all families of people with schizophrenia. The approach emerged from research on **expressed emotion (EE)** — a measure of the family's emotional climate.

Expressed Emotion (Brown, Birley & Wing, 1972)

Component	Description
Critical comments	Negative remarks about the patient's behaviour (tone and content)
Hostility	Generalised negative attitude toward the patient as a person
Emotional overinvolvement (EOI)	Excessive concern, self-sacrificing behaviour, overprotectiveness

High EE (high criticism and/or EOI) predicts relapse: 9–month relapse rates are approximately 50% in high-EE families vs. 20% in low-EE families (Bebbington & Kuipers, 1994).

KEY INSIGHT

MNEMONIC — CHE (Expressed Emotion components): **C**ritical comments, **H**ostility, **E**motional overinvolvement. Two of three (critical comments + hostility OR EOI) = high EE.

Family Intervention Models

Model	Key Features
Kuipers, Leff & Lam (psychoeducational)	Education about schizophrenia, stress-vulnerability model, communication training, problem-solving, relapse prevention; 10+ sessions over 3–9 months
Falloon (behavioural family therapy)	Structured problem-solving and communication skills training at home; more behavioural focus
McFarlane (multifamily groups)	Multiple families meet together; reduces isolation, provides mutual support; problem-solving workshops

EXAM PEARL

Family intervention should be offered for at least 10 sessions over 3–9 months (NICE CG178). It reduces relapse by approximately 20% over standard care. It must include at least one of: psychoeducation, communication training, problem-solving. It does NOT require the patient's attendance at every session.

CLINICAL ANCHOR

High EE is not "bad parenting" — it is an understandable response to living with a person who has a severe and unpredictable illness. Reframing EE as a measure of distress rather than fault is essential for therapeutic engagement with families.

4. CULTURAL CONSIDERATIONS

4.1 Why Culture Matters in Psychotherapy

All psychotherapy occurs within a cultural context. The major psychotherapy models (CBT, psychodynamic, humanistic) were developed predominantly within Western, Educated, Industrialised, Rich, Democratic (WEIRD) societies.

Their assumptions about selfhood, emotional expression, family structure, and the goals of therapy are culturally specific.

Key areas where culture shapes therapy:

- **Concept of self** — individualist (Western: autonomous self, personal goals) vs. collectivist (Asian, African, Latin American: relational self, family/community goals)
- **Emotional expression** — norms about displaying emotion; alexithymia vs. emotional expressiveness; somatisation as culturally patterned idiom of distress
- **Help-seeking** — stigma, explanatory models, preference for traditional/religious healers, trust in medical systems
- **Therapeutic relationship** — expectations about authority, directiveness, self-disclosure, formality
- **Family involvement** — in many cultures, excluding family from treatment is unthinkable

4.2 Kleinman's Explanatory Models

Arthur Kleinman (1980) introduced the concept of **explanatory models** — the beliefs that patients, families, and clinicians hold about the nature, cause, course, and appropriate treatment of illness. Every person brings an explanatory model to the clinical encounter, and mismatches between patient and clinician models are a major source of non-adherence and therapeutic failure.

Eight questions to elicit an explanatory model:

1. What do you call your problem?
2. What do you think caused it?
3. Why do you think it started when it did?
4. What does your illness do to you?
5. How severe is it? Will it last a long time?

6. What do you fear most about your illness?
7. What kind of treatment do you think you should receive?
8. What are the most important results you hope to achieve?

> EXAM PEARL
>
> Kleinman's explanatory model framework is the intellectual ancestor of the DSM-5 Cultural Formulation Interview. Always elicit the patient's explanatory model before imposing a biomedical or psychological framework. A patient who attributes depression to "evil eye" or "karma" will not engage with cognitive restructuring unless the therapist first understands and respects their framework.

4.3 The DSM-5 Cultural Formulation Interview (CFI)

The CFI is a 16–question semi-structured interview included in DSM-5 (APA, 2013) designed to systematically assess the cultural context of psychiatric presentations.

Four Domains of the CFI

Domain	Focus	Sample Questions
Cultural definition of the problem	How the patient understands their problem	"What brings you here today? How would you describe your problem?"
Cultural perceptions of cause, context, and support	Perceived causes, role of cultural identity, social stressors, social supports	"Why do you think this is happening to you? Are there any aspects of your background that are important?"
Cultural factors affecting self-coping and past help-seeking	Barriers to care, past treatment, self-coping, family/community resources	"What have you done on your own to cope? Have you sought help from others?"
Cultural factors affecting current help-seeking	Expectations, preferences, barriers for current treatment	"What kind of help do you think would be most useful? Is there anything about the way services are provided that might be a problem?"

KEY INSIGHT

MNEMONIC — DPCH (CFI domains): **D**efinition of problem, **P**erceptions of cause, **C**oping and past help-seeking, **H**elp-seeking (current).

4.4 Therapy Adaptations Across Cultures

Bernal's ecological validity framework (Bernal et al., 1995) identifies eight dimensions for culturally adapting therapies:

Dimension	Description
Language	Therapy in patient's preferred language; bilingual therapists; interpreters
Persons	Therapist-patient ethnic/cultural match; cultural knowledge
Metaphors	Culturally resonant symbols, stories, proverbs
Content	Cultural knowledge integrated into case formulation
Concepts	Treatment concepts framed in culturally consistent terms
Goals	Aligned with cultural values (e.g., family harmony vs. individual autonomy)
Methods	Adapted techniques (e.g., involvement of elders, spiritual practices)
Context	Social, economic, and political realities acknowledged

CLINICAL ANCHOR

Cultural adaptation does not mean watering down evidence-based therapy. Meta-analyses (Griner & Smith, 2006; Benish et al., 2011) show that culturally adapted therapies produce larger effect sizes than unadapted versions, particularly for depression and anxiety in ethnic minority populations.

4.5 Working with Interpreters

When therapist and patient do not share a language, professional interpreters are essential. Family members should not serve as interpreters (risk of filtering, power dynamics, confidentiality breaches, emotional burden on the family member).

Guidelines:

- Pre-session briefing with interpreter (explain purpose, confidentiality, request verbatim translation)
- Speak to the patient, not the interpreter ("Tell me about..." not "Ask him about...")
- Use short sentences; pause for translation
- Post-session debrief with interpreter (emotional impact, cultural observations)

4.6 Therapist Cultural Competence

Sue and Sue (2015) describe three components of multicultural competence:

1. **Awareness** of own cultural values, biases, and assumptions
2. **Knowledge** of the worldview, history, and values of culturally diverse groups
3. **Skills** — ability to use culturally appropriate interventions

> EXAM STRATEGY
>
> Cultural competence is not a destination — it is an ongoing process. The shift in the literature is now toward "cultural humility" (Tervalon & Murray-Garcia, 1998): a lifelong commitment to self-evaluation, addressing power imbalances, and being a learner rather than an expert on another person's culture.

5. PERINATAL MENTAL HEALTH

5.1 Scope of the Problem

Perinatal mental health covers the period from conception to one year postpartum. Depression affects 10–15% of women in the perinatal period; anxiety disorders are equally prevalent but underrecognised. Untreated perinatal depression is associated with preterm birth, low birth weight, impaired mother-infant bonding, and adverse child developmental outcomes.

5.2 Therapy Approaches

CBT for Perinatal Depression

Adapted CBT addresses:

- Activity scheduling (counteracting withdrawal and fatigue)
- Cognitive restructuring of maternal cognitions ("I'm a bad mother," "My baby doesn't love me")
- Behavioural activation despite sleep deprivation and reduced positive reinforcement
- Problem-solving for practical stressors (childcare, finances, relationship conflict)

Evidence: NICE (CG192, 2014) recommends CBT or IPT as first-line psychological treatments for mild-moderate perinatal depression.

IPT for Perinatal Depression

Interpersonal therapy (IPT) is particularly well-suited to the perinatal period because its four problem areas map directly onto common perinatal stressors:

IPT Problem Area	Perinatal Application
Role transition	Transition to motherhood; identity shift; loss of autonomy
Interpersonal dispute	Partner conflict; renegotiating roles; in-law involvement
Grief	Pregnancy loss, stillbirth, loss of the "expected" birth experience
Interpersonal deficit	Social isolation, loss of work relationships, limited support network

EXAM PEARL

IPT has the strongest evidence for perinatal depression, comparable to CBT. It is the only therapy where the four problem areas map almost perfectly onto the perinatal experience (role transition to motherhood, partner disputes about parenting, grief for the pre-baby self/lifestyle, and social isolation).

Mother-Infant Relationship Interventions

- **Video Interaction Guidance (VIG)** — filming mother-infant interaction and reviewing with the mother, highlighting moments of attuned communication
- **Circle of Security** — attachment-based intervention; helps parents recognise their infant's cues and respond sensitively
- **Parent-Infant Psychotherapy** — psychodynamic approach addressing how the parent's own attachment history (ghosts in the nursery) affects interaction with the baby

CLINICAL ANCHOR

Treating the mother's depression alone is necessary but not sufficient. If the mother-infant relationship has been disrupted, targeted relationship-focused intervention is needed in addition to individual therapy. The infant's developmental window is narrow — delays in bonding have cumulative effects.

6. INTELLECTUAL DISABILITY

6.1 Prevalence and Context

People with intellectual disability (ID) have rates of mental health problems 3–4 times higher than the general population, yet access to psychological therapy is disproportionately low. Historical nihilism ("they can't do therapy") is both empirically wrong and ethically indefensible.

6.2 Adapted CBT

CBT can be effectively adapted for people with mild-moderate ID. Key adaptations:

Adaptation	Rationale
Simplified language	Match to cognitive and linguistic level
Concrete examples	Abstract concepts replaced with visual aids, pictures, role plays
Shorter sessions	30 minutes; more frequent
Repetition	Overlearning through repeated practice
Involve carers	To reinforce skills between sessions; generalisation
Visual aids	Traffic light systems for emotions; faces for mood ratings; picture thought records
Behavioural emphasis	Greater emphasis on behavioural strategies (activity scheduling, graded exposure) than cognitive techniques
Slower pace	More sessions needed; slower progression through treatment

EXAM PEARL

The Royal College of Psychiatrists' CR175 report confirms that psychological therapies can and should be offered to people with intellectual disability, with appropriate adaptations. The key adaptations are: simplification, concreteness, repetition, visual aids, carer involvement, and behavioural emphasis.

6.3 Behavioural Approaches

For people with severe-profound ID, behavioural approaches remain the primary intervention:

- **Functional analysis** — identifying the function of challenging behaviour (attention, escape, sensory stimulation, tangible gain)
- **Positive Behavioural Support (PBS)** — proactive, person-centred approach that modifies the environment and teaches replacement behaviours
- **Active Support** — structured but flexible approach to enabling engagement in meaningful activities

> KEY INSIGHT
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> **MNEMONIC — AEST** (functions of challenging behaviour): **A**ttention, **E**scape, **S**ensory, **T**angible.

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7. TRAUMA AND COMPLEX PTSD

7.1 Phase-Based Treatment

The ICD-11 recognises Complex PTSD (CPTSD) as a distinct diagnosis comprising core PTSD symptoms (re-experiencing, avoidance, hyperarousal) plus Disturbances of Self-Organisation (DSO): affect dysregulation, negative self-concept, and interpersonal difficulties. The International Society for Traumatic Stress Studies (ISTSS, 2019) recommends a phase-based approach for CPTSD:

Three Phases

Phase	Focus	Interventions
Phase 1: Stabilisation	Safety, psychoeducation, affect regulation, building therapeutic alliance	Grounding techniques, distress tolerance skills, emotion regulation, establishing safety, addressing substance use, building coping resources
Phase 2: Trauma Processing	Working through traumatic memories	EMDR, Prolonged Exposure (PE), Cognitive Processing Therapy (CPT), Narrative Exposure Therapy (NET)
Phase 3: Integration and Reconnection	Rebuilding identity, relationships, and meaning	Interpersonal work, vocational rehabilitation, re-establishing life roles, relapse prevention, addressing grief and loss

EXAM PEARL

The phase-based model originates from Judith Herman's "Trauma and Recovery" (1992): Safety → Remembrance and Mourning → Reconnection. The ISTSS guidelines endorse this framework but note that phases are not strictly sequential — patients may need to return to stabilisation during processing.

7.2 Contraindications for Trauma Processing

Trauma processing (Phase 2) should NOT be started when:

- Active suicidality or self-harm
- Ongoing abuse or unsafe living situation
- Severe dissociation (DES score > 30) without stabilisation
- Active substance dependence (not stable recovery)
- Insufficient affect regulation skills
- Acute psychosis

- Inadequate therapeutic alliance

> CLINICAL ANCHOR
>
> Premature exposure to trauma material in an unstabilised patient risks retraumatisation, dissociative crisis, or dropout. The stabilisation phase is not a delay to "real" therapy — it IS therapy. Many complex trauma patients spend months in Phase 1, and some may never need formal trauma processing if stabilisation and skills development produce sufficient recovery.

7.3 Evidence-Based Trauma Therapies

Therapy	Developer	Key Feature	Best Evidence For
Prolonged Exposure (PE)	Edna Foa	Imaginal + in-vivo exposure; emotional processing theory	Single-incident PTSD
Cognitive Processing Therapy (CPT)	Patricia Resick	Written trauma account + cognitive restructuring of stuck points	PTSD including sexual assault
EMDR	Francine Shapiro	Bilateral stimulation during trauma recall; adaptive information processing model	PTSD (NICE-recommended alongside TF-CBT)
Narrative Exposure Therapy (NET)	Neuner & Schauer	Lifeline exercise; contextualise multiple traumas chronologically	Refugees, multiple/sequential trauma

> EXAM STRATEGY
>
> For PTSD in adults, NICE (NG116) recommends trauma-focused CBT (PE or CPT) or EMDR as first-line treatments. Both should be offered for a minimum of 8–12 sessions. Non-trauma-focused therapies (supportive counselling, relaxation) are NOT recommended as stand-alone treatments for PTSD.

8. LGBTQ+ POPULATIONS

8.1 The Minority Stress Model

Ilan Meyer (2003) proposed the **minority stress model** to explain the elevated rates of mental health problems in sexual and gender minority populations. The model identifies distal and proximal stressors:

Stressor Type	Examples
Distal (external)	Discrimination, violence, prejudice events, microaggressions, rejection
Proximal (internal)	Internalised homophobia/transphobia, expectations of rejection, concealment of identity

The stressors are additive and interact with general life stressors. Resilience factors include community connectedness, social support, and positive identity development.

> EXAM PEARL
>
> Meyer's minority stress model is the dominant framework for understanding mental health disparities in LGBTQ+ populations. It explicitly states that elevated rates of depression, anxiety, substance use, and suicidality are NOT caused by being LGBTQ+ per se, but by the chronic stress of living in a stigmatising society. This distinction is critical — it locates the problem in the environment, not the identity.

8.2 Affirmative Therapy Principles

Affirmative therapy is not a separate modality — it is a set of principles that should inform all therapy with LGBTQ+ clients, regardless of the specific approach used (CBT, psychodynamic, systemic, etc.).

Core principles:

1. Sexual orientation and gender identity are normal aspects of human diversity — not pathology
2. The therapist has a responsibility to understand LGBTQ+ lives, culture, and challenges
3. Therapy explores how minority stress contributes to presenting problems
4. The therapist examines their own biases and assumptions
5. The therapist supports the client's self-determination regarding identity, disclosure, and relationships
6. Conversion/reparative therapy is harmful and unethical (condemned by WHO, APA, RCPsych, and all major professional bodies)

> CLINICAL ANCHOR
>
> Affirmative therapy does not mean that every presenting problem is about being LGBTQ+. Many LGBTQ+ clients present with issues unrelated to identity. The therapist holds both truths: (1) being LGBTQ+ may be entirely irrelevant to this particular problem, AND (2) the background radiation of minority stress is always worth checking as a contributing factor.

8.3 Identity-Specific Considerations

Transgender and gender-diverse clients:

- Use chosen name and pronouns without hesitation
- Understand the gender dysphoria assessment process (if relevant) without making all therapy about gender
- Be aware of intersecting stressors: transphobia, employment discrimination, family rejection, medical barriers
- Therapy may address gender identity exploration, transition-related decisions, or unrelated mental health concerns

Bisexual individuals:

- Face "double discrimination" — biphobia from both heterosexual and gay/lesbian communities
- Higher rates of depression, anxiety, and suicidality than gay/lesbian individuals (bisexual invisibility, identity invalidation)
- Therapists should validate bisexual identity without assuming it is a "phase" or "confusion"

Intersectionality:

- LGBTQ+ individuals from ethnic minority backgrounds face compounded minority stress
- Religious LGBTQ+ individuals may face identity conflict requiring sensitive exploration
- Older LGBTQ+ adults may have lived through periods of criminalisation and carry specific cohort trauma

> EXAM STRATEGY
>
> If asked about therapy with LGBTQ+ clients, lead with Meyer's minority stress model, then affirmative therapy principles, then the ethical prohibition on conversion therapy. Mention intersectionality. This demonstrates conceptual depth beyond surface-level cultural sensitivity.

SPECIAL POPULATION x RECOMMENDED

THERAPY MATRIX

Population	First-Line Psychological Therapy	Key Adaptation	Evidence Level
Child anxiety (7–13)	CBT (Coping Cat/Cool Kids)	FEAR steps, graded exposure, parental involvement	Strong (multiple RCTs)
Child conduct disorder (3–11)	Parent training (Webster-Stratton, Triple P, PCIT)	Work primarily with parents; operant principles	Strong (NICE CG158)
Adolescent depression	CBT + SSRI (combination)	TADS model; slower onset for CBT alone	Strong (TADS RCT)
Adolescent self-harm/suicidality	DBT (Rathus-Miller model)	5 modules, multifamily group, 24 weeks	Moderate-Strong (Mehlum RCT)
Late-life depression	CBT (Laidlaw CCCF), IPT, problem-solving therapy	Pace, repetition, sensory accommodations, CRIS factors	Moderate-Strong
Psychosis	CBTp + family intervention	Normalising approach, behavioural experiments, EE reduction	Strong (NICE CG178)
Perinatal depression	CBT or IPT	Role transition focus; mother-infant work	Strong (NICE CG192)
Intellectual disability	Adapted CBT, PBS	Simplification, visual aids, carer involvement, behavioural emphasis	Moderate
PTSD (single-incident)	TF-CBT (PE/CPT) or EMDR	8–12 sessions minimum	Strong (NICE NG116)

Population	First-Line Psychological Therapy	Key Adaptation	Evidence Level
Complex PTSD	Phase-based: stabilisation → processing → integration	Phase 1 may take months; contraindications for premature processing	Moderate (ISTSS guidelines)
LGBTQ+ populations	Any evidence-based therapy + affirmative principles	Minority stress awareness; no conversion therapy	Consensus/Moderate
Cultural minorities	Culturally adapted evidence-based therapy	CFI, explanatory models, language, family involvement	Moderate (meta-analyses)

SUMMARY OF CALLOUTS

Exam Pearls (15)

1. Metacognition emerges at 11–12; CBT difficult before this age
2. Kendall's Coping Cat — FEAR acronym, two-phase structure
3. TADS — combination > fluoxetine > CBT = placebo at 12 weeks
4. PCIT — real-time coaching via earpiece; evidence for abuse prevention
5. TIPP skills — dive reflex via cold water, fastest DBT skill
6. Older adults underreferred for therapy; efficacy comparable to younger adults
7. Life review therapy based on Erikson's ego integrity vs. despair
8. Normalising rationale central to CBTp — continuum model
9. Freeman's paranoia model — JTC bias, safety behaviours, six maintenance factors
10. Family intervention: 10+ sessions, reduces relapse by ~20%

11. Kleinman's explanatory models — ancestor of DSM-5 CFI
12. IPT's four problem areas map perfectly onto perinatal stressors
13. Royal College confirms psychological therapies should be offered to people with ID
14. Phase-based model from Herman (1992): Safety → Remembrance → Reconnection
15. Meyer's minority stress model — problem in the environment, not the identity

Clinical Anchors (7)

1. Play therapy — trained therapist observes themes, not "just playing"
2. Biosocial model — not a blame model; "fire in a fireplace" metaphor
3. CBTp — "curious uncertainty" stance; never directly challenge delusions
4. High EE — reframe as distress, not fault
5. Cultural adaptation produces larger effect sizes than unadapted therapy
6. Mother-infant relationship needs targeted intervention beyond treating maternal depression
7. Affirmative therapy — being LGBTQ+ may be irrelevant to the presenting problem; check minority stress as background

Exam Strategies (4)

1. TADS results by timepoint
2. Match population to first-line therapy (table above)
3. Older adult therapy viva: mention CCCF, life review, cognitive screening, practical barriers
4. LGBTQ+ therapy: Meyer → affirmative principles → conversion therapy prohibition → intersectionality

Mnemonics (4)

1. **WITP** — parent training programmes
2. **CRIS** — Laidlaw's four cohort-specific factors
3. **CHE** — Expressed Emotion components
4. **DPCH** — CFI domains
5. **AEST** — Functions of challenging behaviour in ID

02

Clinical Quick Reference

Psychotherapy for Special Populations — Weave Psychotherapy Vol. 10

D6: Psychotherapy for Special Populations — Quick Reference

1. Child Therapy Modalities by Age

Age	Cognitive Stage	Recommended Modalities	Rationale
0–2	Sensorimotor	Parent-infant psychotherapy, VIG, Circle of Security	Child cannot participate directly; work through parent-infant dyad
2–4	Early preoperational	Non-directive play therapy (Axline), Theraplay, PCIT	Symbolic play as communication; parent coaching
4–7	Late preoperational	CBPT (Knell), directive play therapy, parent training	Can follow simple instructions; limited metacognition
7–11	Concrete operational	CBT (Coping Cat), group CBT, parent training as adjunct	Can classify, follow rules; exposure hierarchies possible
12–17	Formal operational	Standard CBT adapted, DBT (Rathus-Miller), IPT-A	Abstract reasoning; metacognition available; identity work

2. Parent Training Programmes Comparison

Feature	Webster-Stratton (Incredible Years)	Triple P	PMT (Kazdin)	PCIT (Eyberg)
Age range	3–8	0–16	3–13	2–7
Format	Group (12–20 sessions)	5 levels (universal → intensive)	Individual (12–20)	Dyadic, live coaching
Unique feature	Video modelling, collaborative	Population-level public health model	Operant, token economies	Bug-in-ear real-time coaching
Components	Child + parent + teacher pyramid	Level 1–5 escalation	Parent-only sessions	CDI (relationship) + PDI (compliance)
Target	Conduct disorder, ODD	Universal prevention → clinical treatment	Conduct problems	Conduct problems + abuse prevention
Evidence	Strongest for conduct disorder	Strong at population level	Strong	Strong; unique abuse prevention data

3. Adolescent DBT — Key Facts

Feature	Detail
Developers	Jill H. Rathus & Alec L. Miller
Population	Multiproblem, suicidal adolescents
Modules	5: Mindfulness, Distress Tolerance, Walking the Middle Path, Emotion Regulation, Interpersonal Effectiveness
Unique module	Walking the Middle Path (dialectics, validation, behaviour change)
Format	Multifamily group (parents + teens), 2–hour sessions, 3–5 families
Duration	24 weeks (one full cycle)
Modes	Individual therapy, multifamily skills group, phone coaching (teens + parents), therapist consultation team
Key adaptations	Simplified language, developmentally appropriate examples, shorter sessions
Biosocial model	Biological vulnerability (sensitivity + reactivity + slow return) × invalidating environment
Adolescent dilemmas	Leniency vs. authoritarian; normalising pathological vs. pathologising normative; forcing autonomy vs. fostering dependence
Crisis skills	TIPP: Temperature (dive reflex), Intense exercise, Paced breathing, Progressive relaxation
Key RCT	Mehlum et al. (2014) — DBT > enhanced usual care for self-harm

4. CBTp — Key Techniques Checklist

Technique	Target Symptom	Description
Normalising rationale	All psychotic experiences	Continuum model; reduces shame and hopelessness
Shared formulation	Delusions, hallucinations	Develop collaborative understanding in context (stress, trauma, life events)
Peripheral questioning	Delusions	Start with less charged beliefs; work inward
Socratic questioning	Delusions	"What evidence supports this? Is there another explanation?"
Behavioural experiments	Paranoia, delusions	Drop safety behaviours; test predictions
Beliefs about voices work	Hallucinations	Challenge omnipotence, omniscience, compliance beliefs (Chadwick & Birchwood)
Coping strategy enhancement	Hallucinations	Build on existing coping; add distraction, subvocalisation
Reattribution	Hallucinations	Consider internal generation of voices
Activity scheduling	Negative symptoms	Combat withdrawal, apathy, social isolation
Relapse prevention	All	Identify early warning signs; develop action plan

Freeman's 6 maintenance factors for paranoia: anxiety, worry, negative self-beliefs, sleep problems, reasoning biases (JTC), safety behaviours.

5. Expressed Emotion and Family Intervention

Component	Definition	Threshold
Critical comments	Negative remarks about behaviour (tone + content)	≥6 on CFI = high
Hostility	Generalised rejection of the person	Present/absent
Emotional overinvolvement	Overprotection, self-sacrifice, excessive concern	Rated 0–5; ≥3 = high

High EE = elevated critical comments/hostility OR high EOI → ~50% 9–month relapse rate (vs. ~20% low EE).

NICE recommendation: Family intervention for all with schizophrenia; ≥10 sessions over 3–9 months; includes psychoeducation + communication training + problem-solving.

6. Cultural Formulation Interview (CFI) — Steps

Step	Domain	Key Questions
1	Cultural definition of problem	"What brings you here? How would you describe your problem? What do you call it?"
2	Cultural perceptions of cause	"Why do you think this is happening? Any aspects of background/identity relevant?"
3	Stressors and supports	"What stresses make it worse? Who/what helps?"
4	Role of cultural identity	"Is your cultural background important to this problem? How?"
5	Past coping and help-seeking	"What have you done on your own? Sought help from traditional healers/religious leaders?"
6	Current help-seeking	"What kind of help would be most useful? Barriers to treatment?"

Kleinman's 8 questions for explanatory models: name, cause, timing, mechanism, severity/duration, fears, expected treatment, hoped results.

Bernal's 8 adaptation dimensions: Language, Persons, Metaphors, Content, Concepts, Goals, Methods, Context.

7. Phase-Based Trauma Treatment

Phase	Goal	Interventions	Duration
1: Stabilisation	Safety, affect regulation, coping	Grounding, distress tolerance, psychoeducation, substance use, therapeutic alliance	Weeks to months
2: Processing	Work through traumatic memories	PE, CPT, EMDR, NET	8–12+ sessions
3: Integration	Rebuild identity, relationships, meaning	Interpersonal work, vocational rehab, relapse prevention, grief work	Open-ended

Contraindications for Phase 2: active suicidality, ongoing abuse, severe dissociation (DES >30), active substance dependence, insufficient affect regulation, acute psychosis, poor therapeutic alliance.

Trauma therapy comparison:

Therapy	Key Mechanism	Best For
Prolonged Exposure (Foa)	Imaginal + in-vivo exposure	Single-incident PTSD
CPT (Resick)	Trauma account + stuck points	PTSD including sexual assault
EMDR (Shapiro)	Bilateral stimulation + AIP model	PTSD (NICE first-line)
NET (Neuner/Schauer)	Lifeline, chronological contextualisation	Refugees, multiple traumas

8. Special Populations x Therapy Matrix

Population	First-Line Therapy	Key Evidence
Child anxiety (7–13)	CBT (Coping Cat)	60–80% remission; Kendall RCTs
Child conduct (3–11)	Parent training	NICE CG158
Adolescent depression	CBT + SSRI	TADS (2004)
Adolescent self-harm	DBT (Rathus-Miller)	Mehlum et al. (2014)
Late-life depression	CBT (CCCF model), IPT	Laidlaw (2015)
Psychosis	CBTp + family intervention	NICE CG178
Perinatal depression	CBT or IPT	NICE CG192
Intellectual disability	Adapted CBT + PBS	RCPsych CR175
PTSD	TF-CBT or EMDR	NICE NG116
Complex PTSD	Phase-based treatment	ISTSS (2019)
LGBTQ+	Any EBT + affirmative principles	Meyer (2003)

9. Key Mnemonics

Mnemonic	Stands For	Context
FEAR	Feeling frightened, Expecting bad things, Actions to help, Results and rewards	Coping Cat (child anxiety CBT)
TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation	Adolescent DBT crisis skills
CRIS	Cohort beliefs, Role transitions, Intergenerational linkages, Socio-cultural context	Laidlaw's CCCF for older adults
CHE	Critical comments, Hostility, Emotional overinvolvement	Expressed Emotion components
DPCH	Definition of problem, Perceptions of cause, Coping/help-seeking (past), Help-seeking (current)	CFI domains
AEST	Attention, Escape, Sensory, Tangible	Functions of challenging behaviour (ID)
WITP	Webster-Stratton, Incredible Years, Triple P, PCIT/PMT	Parent training programmes

10. Viva Questions

Q1. What are the key adaptations needed when delivering CBT to a 7-year-old child?

Shorter sessions (30–45 min), simplified language, concrete examples (not abstract thought records), visual aids, behavioural emphasis over cognitive techniques, parental involvement for reinforcement, play-based delivery where appropriate, and developmental stage-appropriate content. Metacognition is limited before age 11–12, so focus on behavioural experiments and exposure rather than cognitive restructuring.

Q2. Describe the biosocial model of adolescent DBT.

The biosocial model explains chronic emotional dysregulation through the transactional interaction between biological vulnerability (high emotional sensitivity, high reactivity, slow return to baseline) and a pervasively invalidating environment (punishing emotion, reinforcing escalation, oversimplifying solutions). The model is transactional — both teen and family contribute. Rathus and Miller added three adolescent-specific dialectical dilemmas: leniency vs. authoritarian control, normalising pathological vs. pathologising normative, forcing autonomy vs. fostering dependence.

Q3. What is Laidlaw's CCCF model and why is it needed?

The Comprehensive Conceptualisation Framework adds four cohort-specific factors to the standard CBT formulation for older adults: Cohort beliefs (generational attitudes), Role transitions (retirement, widowhood), Intergenerational linkages (grandparenting, legacy), and Socio-cultural context (ageism, isolation). It is needed because standard CBT formulations may pathologise normal ageing or miss social determinants unique to later life.

Q4. How does CBTp approach working with delusions?

CBTp uses the normalising rationale (continuum model) and collaborative Socratic exploration rather than direct confrontation. Steps: build a shared formulation, identify maintaining factors (safety behaviours, confirmation bias), use peripheral questioning (start with less charged beliefs), examine evidence collaboratively, design behavioural experiments to test delusional predictions, and gentle reality testing. The stance is "curious uncertainty" — neither agreeing nor arguing.

Q5. What is expressed emotion, and how does it affect schizophrenia relapse?

Expressed emotion (EE) comprises critical comments, hostility, and emotional overinvolvement. High EE families have ~50% 9–month relapse rates vs. ~20% in low EE families. Family intervention targeting EE reduction (psychoeducation, communication training, problem-solving) is recommended by NICE for at least 10 sessions over 3–9 months and reduces relapse by approximately 20%.

Q6. Describe the DSM-5 Cultural Formulation Interview.

The CFI is a 16–question semi-structured interview covering four domains: cultural definition of the problem, cultural perceptions of cause/context/support, cultural factors affecting past coping and help-seeking, and cultural factors affecting current help-seeking. It is based on Kleinman's explanatory model framework and aims to systematically assess cultural context to improve diagnostic accuracy and treatment engagement.

Q7. What is the phase-based approach to Complex PTSD?

Three phases: (1) Stabilisation — safety, affect regulation, therapeutic alliance, grounding, distress tolerance; (2) Trauma processing — PE, CPT, EMDR, or NET; (3) Integration — rebuilding identity, relationships, and meaning. Contraindications for Phase 2 include active suicidality, ongoing abuse, severe dissociation, active substance dependence, and insufficient affect regulation. Based on Herman (1992).

Q8. What is Meyer's minority stress model?

Meyer (2003) explains LGBTQ+ mental health disparities through distal stressors (discrimination, violence, prejudice) and proximal stressors (internalised homophobia/transphobia, concealment, expectations of rejection). Elevated rates of depression, anxiety, and suicidality are attributed to chronic minority stress, not to sexual/gender identity itself. This model informs affirmative therapy principles.

Q9. What therapy is recommended for perinatal depression and why is IPT particularly suitable?

NICE recommends CBT or IPT as first-line for mild-moderate perinatal depression. IPT is particularly suitable because its four problem areas map directly onto perinatal stressors: role transition (to motherhood), interpersonal disputes (partner conflict about parenting), grief (pregnancy loss or loss of pre-baby identity), and interpersonal deficits (social isolation). Mother-infant relationship interventions (VIG, Circle of Security) may also be needed.

Q10. How should CBT be adapted for people with intellectual disability?

Key adaptations: simplified language matched to cognitive level, concrete examples with visual aids (traffic lights, picture thought records), shorter sessions (30 min), more frequent sessions, greater repetition and overlearning, stronger behavioural emphasis over cognitive techniques, carer involvement for generalisation, and slower overall pace. Functional analysis and positive behavioural support are the primary approaches for severe-profound ID.

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